

Costimulatory Blockade After Initial Biologic Response in Psoriasis: Relapse Prevention and Long-Term Disease Control

Hana Moreau, Jana Farrell, Bruno Toivonen, Oleksandr Adeyemi, Aisling Okoye

ABSTRACT

Patients with psoriasis who respond favorably to biologic therapy face uncertainty regarding optimal long-term disease management strategies. We evaluated whether switching to an alternative costimulatory-blocking biologic agent could prevent disease relapse following withdrawal of an initial biologic therapy. One hundred eight adults with moderate-to-severe psoriasis received lead-in therapy with an established biologic agent, and 91 responders were subsequently randomized to either continue the original biologic or transition to costimulatory blockade, with both withdrawn at week 39. Over the subsequent observation period through week 88, approximately 91% of participants assigned to the alternative costimulatory-blocking agent experienced psoriasis relapse compared to 87% in the continued-therapy group—a difference that was not statistically significant. Median time from treatment discontinuation to disease relapse was similar in both groups (approximately 5 weeks). Molecular assessment of lesional tissue revealed that costimulatory blockade did not adequately suppress the pathogenic inflammatory cascade responsible for psoriasis. Safety profiles remained acceptable in both treatment arms. These findings indicate that costimulatory blockade alone is insufficient to maintain control after withdrawal of the original biologic therapy.

Trial Context

Underlying double-blind randomized controlled trial comparing continued therapy with established biologic agent with costimulatory blockade with abatacept after prior biologic therapy.